

Criteria for Death or Withholding Resuscitation

Standard:

Define the parameters in which providers in the ATCEMS System may withhold resuscitative efforts.

Purpose:

Resuscitation, including CPR, is to be withheld only if the patient is obviously dead per the criteria below or has a valid OOH DNR.

If you are unsure whether the patient meets the criteria, resuscitate.

Application:

Resuscitation efforts should not be initiated or continued by an ATCEMS System Provider if the patient is pulseless and apneic and one or more of the following is present. Document in the ePCR the specific indications for withholding or terminating resuscitation.

1. If the patient meets any of the below criteria, any resuscitative care was not continued or initiated by System Credentialed Providers, a $\geq$ PL2 may contact communications for a time of death:
 1. **Decomposition or Putrefaction** – skin bloated or ruptured, with or without soft tissue sloughed off.
 2. **Decapitation** – the complete severing of the head from the remainder of the patient's body.
 3. **Incineration** – 90% or more of body surface area burned with full thickness burns as exhibited by ash rather than clothing and complete absence of body hair with charred skin
 4. **Obviously mortal wounds** – such as massive crush injury, complete exsanguination, severe displacement of brain matter, or complete transection of the torso.
 5. **Patient submersion** – greater than 20 minutes from the time the patient was witnessed going underwater, from the 9-1-1 call, or from the arrival of the first public safety entity until the patient is in a position for effective resuscitative efforts to begin.
2. If the patient meets any of the below criteria and resuscitative care was not continued or initiated by System Credentialed Providers:
 1. Patient with blunt or penetrating trauma is apneic and pulseless without other signs of life upon EMS arrival
 2. Nontraumatic arrest with dependent lividity
 3. Nontraumatic arrest with rigor mortis involving major joints such as the shoulders, elbows, hips, or knees.

A **BLS** provider may contact communications for a time of death after ensuring:

- a) Pupils are fixed and dilated **and**
- b) Carotid or femoral pulselessness for 30 seconds **and**
- c) Absence of apical heart sounds for 30 seconds

An **ALS** provider may contact communications for a time of death after ensuring:

- a) All requirements under "BLS provider" are fulfilled **or**
- b) Asystole for no less than 6 seconds in 2 or more leads **or**
- c) Cardiac standstill via ultrasound

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Fetal death with a fetus < 20 weeks by best age determination available at the scene is considered products of conception and does not require time of death. Fetal death < 20 weeks may be documented on the mother's ePCR. If $\geq$ 20 weeks, then create a separate ePCR.

If resuscitation efforts have been initiated or continued by a System Credentialed Provider/Responder, then discontinuation is at the discretion of the arriving $\geq$ PL5 provider. In this case, continue resuscitation, and OLMC must be contacted for Termination of Resuscitation (TOR).

If a valid out-of-hospital Do Not Resuscitate (DNR) order is presented or found anytime during ongoing resuscitative attempts, the providers/responders may immediately stop the resuscitation efforts, and a time of TOR may be obtained from communications.

References: [Texas Health and Safety Code Sec 773.016](#); [DSHS Rule 157.25 Out-of-Hospital Do Not Resuscitate \(DNR\) Order](#)